Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0008	**Age / Sex**	51 / F
Setting	Emergency Dept	**Primary dx**	Aneurysmal subarachnoid hemorrhage

Chief complaint

Sudden severe headache, described as the worst of her life.

History of present illness

Onset at 18:10 while lifting groceries: thunderclap occipital headache, vomiting twice, brief collapse without convulsion. Photophobia and neck stiffness since. Daughter states she has become progressively more confused.

Past history / medications

Smoker 25 pack-years, hypertension. Maternal aunt died of a brain hemorrhage. Ramipril.

Examination

BP 190/104, HR 78, afebrile. Meningism present. Left third nerve palsy with ptosis and dilated pupil. No limb weakness.

Glasgow Coma Scale

Eyes open to voice. Verbal confused: disoriented to time and place. Motor: follows commands.

GCS on arrival	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**13**

Investigations

CT head within 3 h of onset: diffuse basal cistern and sylvian fissure subarachnoid blood, Fisher grade 3, mild hydrocephalus. CTA: 7 mm left posterior communicating artery aneurysm.

Assessment

Aneurysmal SAH, Hunt-Hess grade 3 and WFNS grade 2 (GCS 13, no motor deficit). Left PCom aneurysm explaining the oculomotor palsy.

Plan / disposition

BP control (target systolic 140-160), nimodipine 60 mg q4h, analgesia, antiemetics. Neurosurgery and neurointerventional: endovascular coiling within 24 h. Admitted to neuro-ICU for vasospasm surveillance.